Lecture 9: Hypothesis Testing

p-Values · Power Analysis · Permutation Tests · Multiple Testing

Previously, on Lecture 8...

Confidence interval: A random interval $[L, U]$ with $P(L \leq \theta \leq U) = 1 - \alpha$. The *procedure* is 95%.

Wald CI: $\hat{\theta} \pm z_{\alpha/2} \cdot \text{SE}$. Works when CLT kicks in. Use Wilson for proportions.

t-interval: Use when σ unknown and n small. Heavier tails $\Rightarrow$ wider CI.

Bootstrap: Resample with replacement $\Rightarrow$ SE and CI without formulas. BCa is the gold standard.

Two paths: Analytical when formulas exist; bootstrap when they don't. Same goal.

Today: We can estimate and quantify uncertainty. Now: is the effect **real** or **noise**?

Part I: The Logic of Testing

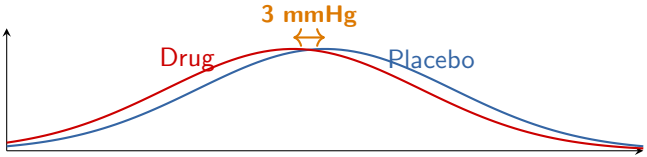
Is 3 mmHg a real effect or just noise?

The Drug Trial

Scenario: A pharmaceutical company tests a new blood pressure drug.

- $n = 100$ patients per group (drug vs. placebo)
- Placebo group mean: $\bar{X}_P = 140$ mmHg
- Drug group mean: $\bar{X}_D = 137$ mmHg
- Pooled SD: $S = 12$ mmHg

Question: Is this 3 mmHg reduction *real*, or could random chance produce it?



Innocent Until Proven Guilty

Courtroom

1. **Assume** defendant is innocent
2. **Examine** the evidence
3. **Convict** only if evidence is overwhelming

"Innocent until proven guilty"

Verdict: guilty / *not proven guilty*
(never "proven innocent")

Statistics

1. **Assume** H_0: no effect
2. **Compute** how surprising the data is
3. **Reject** H_0 only if the data is extremely unlikely under H_0

"No effect until proven otherwise"

Verdict: reject H_0 / *fail to reject* H_0
(never "H_0 is true")

H_0 and H_1

H_0 (null hypothesis)

"Nothing is happening"

The default / boring explanation

Examples:
Drug has no effect: $\mu_D = \mu_P$
The coin is fair: $p = 0.5$
Treatment A = Treatment B

H_1 (alternative)

"Something is happening"

What we want evidence *for*

One-sided: $\mu_D < \mu_P$
(drug lowers BP)
Two-sided: $\mu_D \neq \mu_P$
(drug changes BP)

Key: We never "prove" H_0. We either **reject** it (strong evidence against) or **fail to reject** it (insufficient evidence). Absence of evidence $\neq$ evidence of absence.

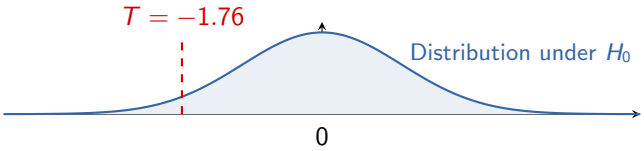
The Test Statistic

A **test statistic** T compresses the data into one number measuring "distance from H_0".

$$T = \frac{\bar{X}_D - \bar{X}_P}{\text{SE}} = \frac{\bar{X}_D - \bar{X}_P}{S\sqrt{2/n}}$$

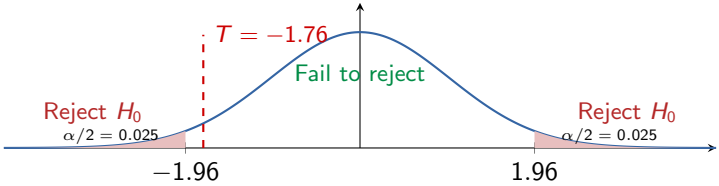
$$\text{Drug trial:} \quad T = \frac{137 - 140}{12\sqrt{2/100}} = \frac{-3}{1.70} = -1.76$$

Under H_0: $T \sim N(0, 1)$ approximately. Large $|T|$ = data far from H_0 = evidence against H_0.



The Rejection Region

Choose a **significance level** α (typically 0.05) *before* looking at the data.

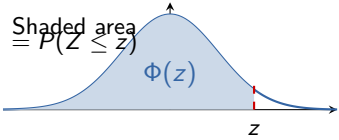


Drug trial: $|T| = 1.76 < 1.96 \Rightarrow$ **fail to reject** H_0 at $\alpha = 0.05$.

The 3 mmHg reduction is *not statistically significant* at the 5% level.

Reading the z-Table: How It Works

Standard Normal cumulative distribution: $\Phi(z) = P(Z \leq z)$



The table gives $\Phi(z)$: the area to the **left** of z. Row = first digit, column = second decimal.

z	.00	.01	.02	.03	.04	.05	.06	.07	.08
1.5	.9332	.9345	.9357	.9370	.9382	.9394	.9406	.9418	.9429
1.6	.9452	.9463	.9474	.9484	.9495	.9505	.9515	.9525	.9535
1.7	.9554	.9564	.9573	.9582	.9591	.9599	.9608	.9616	.9625
1.8	.9641	.9649	.9656	.9664	.9671	.9678	.9686	.9693	.9699
1.9	.9713	.9719	.9726	.9732	.9738	.9744	.9750	.9756	.9761
2.0	.9772	.9778	.9783	.9788	.9793	.9798	.9803	.9808	.9812

Row 1.7 + Col .06

Read: $z = 1.76 \Rightarrow$ row **1.7**, column **.06** $\Rightarrow \Phi(1.76) = 0.9608$.

Using the z-Table: Two Key Tasks

Task 1: Find a p-value

Given $T = 1.76$, find $P(Z > 1.76)$.

Step 1: Look up $\Phi(1.76) = 0.9608$

Step 2: Right tail:
$P(Z > 1.76) = 1 - 0.9608 = 0.0392$

Step 3: For two-sided test:
$p = 2 \times 0.0392 = \mathbf{0.0784}$

(Multiply by 2 because extreme values in *either* tail count as evidence.)

Task 2: Find a critical value

For $\alpha = 0.05$ (two-sided), find $z_{\alpha/2}$.

Step 1: We need $P(Z > z) = 0.025$,
i.e. $\Phi(z) = 1 - 0.025 = 0.975$

Step 2: Scan the table for 0.9750

Step 3: Found at row **1.9**, col **.06**:
$\Phi(1.96) = 0.9750$
$\Rightarrow z_{0.025} = \mathbf{1.96}$

(This is where the famous 1.96 comes from!)

Common critical values: $z_{0.05} = 1.645$ (90% CI) $z_{0.025} = 1.960$ (95% CI) $z_{0.005} = 2.576$ (99% CI)

The Student's t-Distribution

William Sealy Gosset, head brewer at Guinness, Dublin, 1908

The problem

Gosset tested barley and hops with **tiny samples** ($n = 3$–4).

He noticed: using S instead of σ in $T = \frac{\bar{X}-\mu}{S/\sqrt{n}}$ made the distribution **wider** than $N(0,1)$.

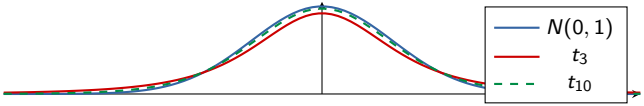
With small n, S is a noisy estimate of σ — extra uncertainty!

The solution

He derived the exact distribution: the t-distribution with df $= n - 1$.

Why "Student"?
Guinness banned employees from publishing. Gosset published under the pseudonym **"Student"** in *Biometrika* (1908).

The name stuck forever.



z vs t: When to Use Which

When σ is **known**: $$T = \frac{\bar{X} - \mu_0}{\sigma/\sqrt{n}} \sim N(0,1) \quad \Rightarrow \text{use } z\text{-table}$$

When σ is **unknown**: $$T = \frac{\bar{X} - \mu_0}{S/\sqrt{n}} \sim t_{n-1} \quad \Rightarrow \text{use } t\text{-table}$$

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When σ is **unknown**: $$T = \frac{\bar{X} - \mu_0}{S/\sqrt{n}} \sim t_{n-1} \quad \Rightarrow \text{use } t\text{-table}$$

In practice:

σ is almost never known!

So we almost always use t.

The drug trial used z because $n = 100$ is large enough that $t_{99} \approx N(0,1)$.

Rule of thumb:

$n \geq 30$: t and z give similar results

$n < 30$: t gives wider intervals (correctly accounts for uncertainty)

$n < 10$: t is essential!

Reading the t-Table

Organized differently from z-table: rows = df, columns = tail probabilities

	Upper-tail probability $P(T > t)$				
df	0.100	0.050	0.025	0.010	0.005
3	1.638	2.353	3.182	4.541	5.841
5	1.476	2.015	2.571	3.365	4.032
10	1.372	1.812	2.228	2.764	3.169
24	1.318	1.711	2.064	2.492	2.797
30	1.310	1.697	2.042	2.457	2.750
60	1.296	1.671	2.000	2.390	2.660
120	1.289	1.658	1.980	2.358	2.617
∞	1.282	1.645	1.960	2.326	2.576

Row 24, Col 0.025

Coffee shop: $n = 25$, **df** $= 24$

For 95% two-sided test:
$\alpha/2 = 0.025$ column
$\Rightarrow t_{0.025,24} =$ **2.064**

Compare: $z_{0.025} = 1.960$

Key observations

df $\uparrow \Rightarrow$ critical value $\downarrow$
(more data = less uncertainty)

df = 3: $t_{0.025} = 3.182$ (huge!)
df $= \infty$: $t_{0.025} = 1.960 = z_{0.025}$

Part II: Two Ways to Be Wrong

False alarms and missed discoveries

Type I and Type II Errors

	Fail to reject H_0	**Reject H_0**
H_0 true	**Correct** True negative $P = 1 - \alpha$	**Type I Error** False alarm $P = \alpha$
H_0 false	**Type II Error** Missed detection $P = \beta$	**Correct** Power $= 1 - \beta$

Type I and Type II Errors

	Fail to reject H_0	**Reject H_0**
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H_0 false	**Type II Error** Missed detection $P = \beta$	**Correct** Power $= 1 - \beta$

Type I (false positive):
Telling a man he's pregnant.
Convict an innocent person.
Approve a useless drug.

Type II (false negative):
Telling a pregnant woman she's not.
Free a guilty person.
Miss an effective drug.

Part III: The p-Value

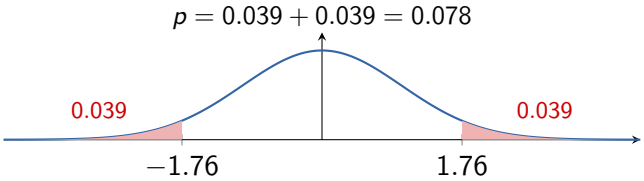
The most used — and most abused — number in science

The p-Value: Definition

The **p-value** is the probability of observing data *this extreme or more*,

assuming H_0 is true:

$$p = P\big(|T| \geq |T_{\text{obs}}| \;\big|\; H_0\big)$$



Drug trial: $p = 0.078 > 0.05 = \alpha \;\Rightarrow\;$ fail to reject H_0.
Rule: Reject H_0 if and only if $p \leq \alpha$.

What the p-Value Is NOT

✗ "$p = 0.03$ means there's a 3% chance H_0 is true."

Reality: $p = P(\text{data} \mid H_0)$, not $P(H_0 \mid \text{data})$. The transpose fallacy!

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Reality: p says nothing about $P(H_1)$. For that you need Bayes.

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✗ "Smaller p = bigger effect."
Reality: p depends on *both* effect size and n. With $n = 10^6$, any tiny effect gives $p \approx 0$.

Statistical vs Practical Significance

With enough data, *any* nonzero effect becomes "statistically significant."

Scenario A

$n = 100{,}000$ per group
Drug lowers BP by **0.1 mmHg**
$p < 0.001$

Statistically significant
Clinically meaningless

Would you take a pill every day for 0.1 mmHg?

Scenario B

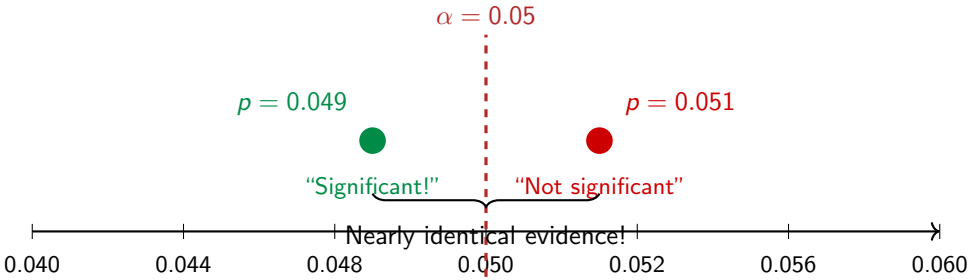
$n = 15$ per group
Drug lowers BP by **8 mmHg**
$p = 0.12$

Not significant
Clinically important

Study was underpowered, not the drug's fault.

Always report: **(1) effect size**, **(2) confidence interval**, **(3) p-value**.
Ask "**How big** is the effect?", not just "Is it nonzero?"

$p = 0.049$ vs $p = 0.051$



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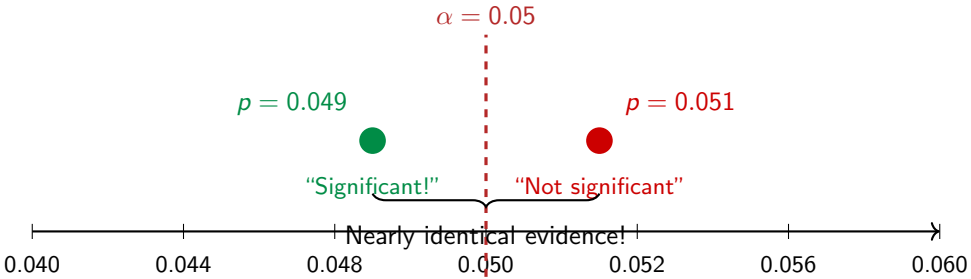


The 0.05 threshold is a **convention**, not a law of nature.

ASA (2016): "Scientific conclusions should not be based only on whether a p-value passes a specific threshold."

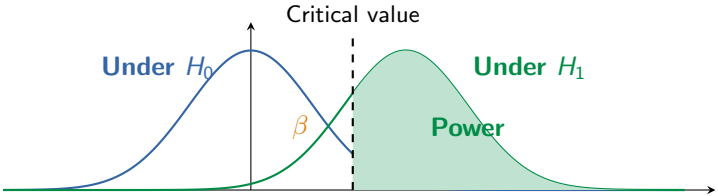
Part IV: Power Analysis

How to design a study that can actually find what you're looking for

Power: The Ability to Detect Real Effects

$$\text{Power} = P\big(\text{reject } H_0 \;\big|\; H_1 \text{ is true}\big) = 1 - \beta$$

Convention: aim for power $\geq$ 0.80 (80%).



What Determines Power?

1. Effect size $\delta \uparrow$

Bigger real effect
$\Rightarrow$ easier to detect
$\Rightarrow$ higher power

2. Sample size $n \uparrow$

More data $\Rightarrow$ narrower SE
$\Rightarrow$ distributions separate
$\Rightarrow$ higher power

3. Significance level $\alpha \uparrow$

Larger $\alpha \Rightarrow$ bigger
rejection region
$\Rightarrow$ higher power (but more Type I)

4. Variability $\sigma \downarrow$

Less noise $\Rightarrow$ clearer signal
$\Rightarrow$ narrower distributions
$\Rightarrow$ higher power

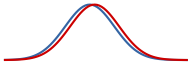
You control n and α. Nature controls δ and σ.
Power analysis = choosing n so that you can detect a meaningful δ.

Effect Size: Cohen's d

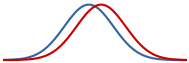
$$d = \frac{\mu_1 - \mu_2}{\sigma}$$

Effect in units of standard deviations.

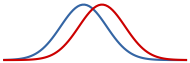
$d = 0.2$ (small)



$d = 0.5$ (medium)



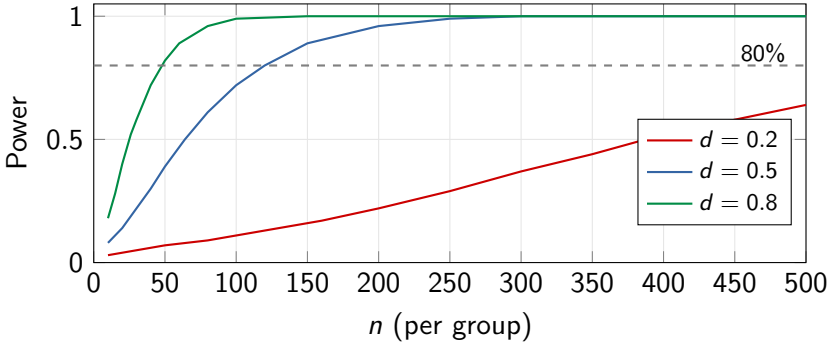
$d = 0.8$ (large)



Drug trial: $d = 3/12 = 0.25$ (small).
Effect size is **independent of sample size** — it measures the *phenomenon*, not the study.

Power Curves: Planning Your Study

Two-sample z-test, $\alpha = 0.05$, two-sided



Required n per group for 80% power: $d = 0.8$: $n \approx 26$ $d = 0.5$: $n \approx 64$ $d = 0.2$: $n \approx 394$

Sample Size Formula

$$n_{\text{per group}} = \frac{2\left(z_{\alpha/2} + z_{\beta}\right)^2}{d^2}$$

where $d = \delta/\sigma$ and $z_{\beta} = 0.84$ for 80% power.

Large effect
$d = 0.8$
$n = 26$

Medium effect
$d = 0.5$
$n = 64$

Small effect
$d = 0.2$
$n = 394$

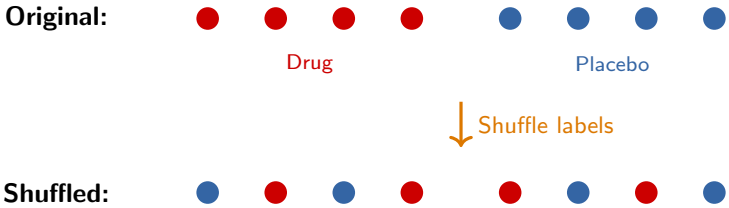
An underpowered study wastes resources. If you can only recruit 50 patients,
you can only detect $d \geq 0.57$. Know this *before* you start collecting data.

Part V: Permutation Tests

No distributional assumptions — let the data speak

If H_0 Is True, Labels Don't Matter

Under H_0: drug and placebo groups come from the *same* distribution.
So the group labels are **arbitrary** — shuffling them shouldn't change anything.

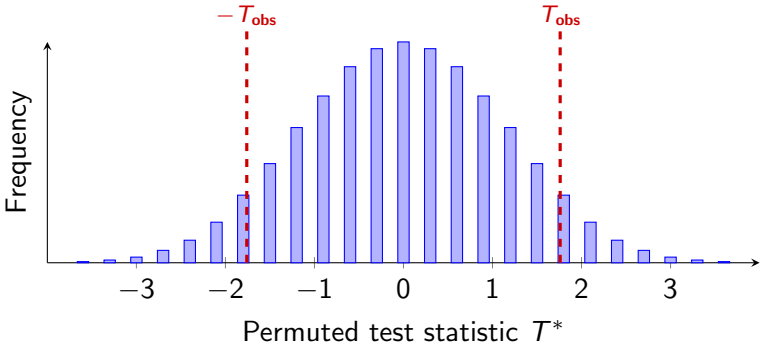


Algorithm:

1. Compute T_{obs} from the original data
2. Shuffle the group labels randomly (keeping group sizes fixed)
3. Compute T^* on the shuffled data
4. Repeat B times (e.g., $B = 10{,}000$)
5. p-value $= \frac{\#\{|T^*| \geq |T_{\text{obs}}|\}+1}{B+1}$

Permutation Distribution

Drug trial: $T_{\text{obs}} = -1.76$, $B = 10{,}000$ permutations



Count: ≈ 780 of 10,000 permutations have $|T^*| \geq 1.76$.
Permutation p-value: ≈ 0.078 — nearly identical to the z-test $p = 0.078$.

Permutation Tests: Pros and Cons

Advantages

+ No distributional assumptions
+ Exact p-values (not approximate)
+ Works for *any* test statistic (median, ratio, custom metric)
+ Easy to understand and implement

Limitations

– Computationally intensive (but cheap on modern machines)
– Tests the "sharp null" only (H_0: identical distributions)
– Cannot easily give CIs (use bootstrap for that)
– Needs exchangeability under H_0

Part VI: Multiple Testing

Test 20 hypotheses, and one will be "significant" by chance

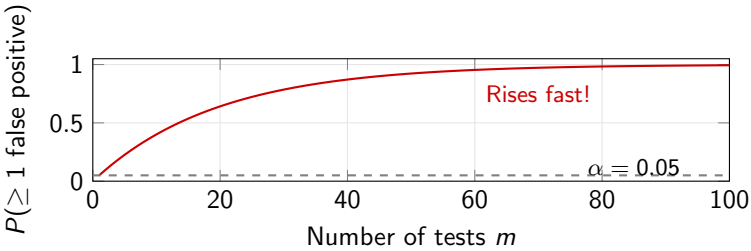
The Multiple Testing Problem

Examples: 20,000 genes, 50 A/B test variants, multiple clinical endpoints

If you test $m = 20$ independent true nulls at $\alpha = 0.05$:

$$P(\text{at least one false positive}) = 1 - (1 - \alpha)^m = 1 - 0.95^{20} = \mathbf{0.64}$$

64% chance of a false discovery — even when nothing is real!



Correcting for Multiple Tests

Bonferroni (FWER)

Reject if $p_i \leq \alpha/m$

For $m = 20$: threshold $= 0.0025$

Controls: $P(\text{any false positive}) \leq \alpha$

+ Simple and safe
– Very conservative when m is large

Benjamini–Hochberg (FDR)

Sort p-values: $p_{(1)} \leq \cdots \leq p_{(m)}$
Find largest k: $p_{(k)} \leq \frac{k}{m}\alpha$
Reject $H_{(1)}, \ldots, H_{(k)}$

Controls: expected *fraction* of false discoveries $\leq \alpha$

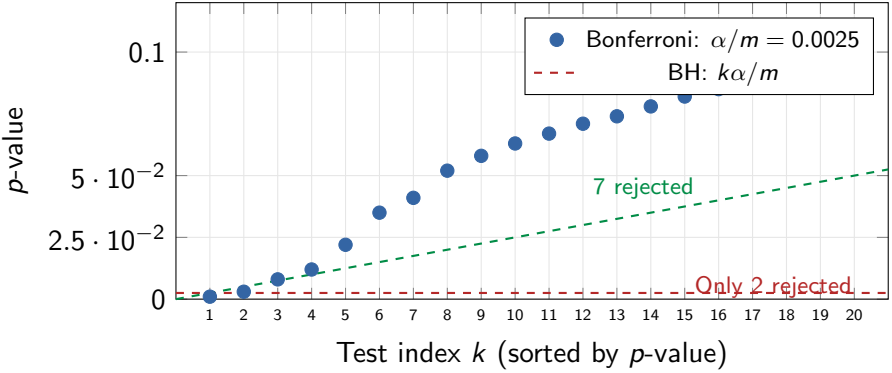
+ More powerful — the modern standard

FWER (Bonferroni): "Every single rejection must be real." Use for small m.

FDR (BH): "Some false discoveries OK if I find more true ones." Use for large m.

BH in Action: The Step-Up Procedure

$m = 20$ tests, $\alpha = 0.05$



Dots below a line = rejected by that method.
BH rejects 7 hypotheses; Bonferroni only 2. BH finds more while controlling the *fraction* of false discoveries.

Summary: Hypothesis Testing

Logic: Assume H_0 (nothing happening). Ask: how surprising is my data under H_0?

Test statistic: Compresses data into one number measuring distance from H_0.

p-value: $P(\text{this extreme or more} \mid H_0)$. **NOT** $P(H_0 \mid \text{data})$.

Type I (α): False alarm. **Type II (β):** Missed detection. **Power** $= 1 - \beta$.

Effect size: Cohen's d measures "how big?" independently of sample size.

Power analysis: Plan n before collecting data. Underpowered studies waste resources.

Permutation tests: Shuffle labels under H_0. No distributional assumptions needed.

Multiple testing: Many tests $\Rightarrow$ many false positives. Bonferroni (strict) or BH-FDR (modern).

Practical: Hypothesis Testing & Power

1. **z-test by hand:**
   - Drug trial: $n = 100$, $\bar{X}_D = 137$, $\bar{X}_P = 140$, $S = 12$
   - Compute T, find p-value, decide at $\alpha = 0.05$
2. **Power simulation:**
   - Under H_1: $\delta = 3$, $\sigma = 12$. Simulate 10,000 experiments
   - Try $n = 100$, 250, 500. What fraction reject H_0?
3. **Permutation test:**
   - With the drug trial data, implement a permutation test ($B = 10{,}000$)
   - Compare p_{perm} with $p_{\text{z-test}}$
4. **Multiple testing:**
   - Simulate 1,000 null experiments ($\mu_1 = \mu_2$). How many give $p < 0.05$?
   - Apply Bonferroni and BH. How many survive each correction?

Homework

1. A coffee shop claims cups contain 350 mL. You measure $n = 25$: $\bar{X} = 345$, $S = 10$.
   Test $H_0 : \mu = 350$ vs $H_1 : \mu \neq 350$ at $\alpha = 0.05$.
   Compute T, p-value, and 95% CI. Does the CI agree with the test?

2. A researcher wants to detect $d = 0.3$ with 80% power at $\alpha = 0.05$.
   (a) How large a sample per group?
   (b) If budget allows only $n = 50$ per group, what is the minimum detectable d?

3. You test 100 genes for disease association. 7 give $p < 0.05$.
   (a) How many false positives do you expect if all nulls are true?
   (b) Apply Bonferroni: how many survive?
   (c) Apply BH at $q = 0.10$: how many survive?

4. Two teaching methods. A scores: 78, 85, 91, 72, 88. B scores: 65, 70, 82, 68, 74.
   Implement a permutation test for the difference in means ($B = 10{,}000$).
   Is the difference significant at $\alpha = 0.05$?

Recommended Visualizations & Resources

Interactive: Seeing Theory — Frequentist Inference (Brown)

`seeing-theory.brown.edu/frequentist-inference` — drag sliders for rejection regions, Type I/II errors, power.

Interactive: R Psychologist — Understanding Statistical Power

`rpsychologist.com/d3/nhst/` — the best interactive power visualization. Drag effect size and n.

Video: StatQuest — p-Values, Clearly Explained

The clearest short explanation of what p-values are and are not. Also: "Hypothesis Testing and p-Values."

Reading: ASA Statement on p-Values (2016)

Six principles every scientist should know. `doi.org/10.1080/00031305.2016.1154108`

Reading: Wasserman, "All of Statistics" — Ch. 10

Hypothesis testing, Neyman–Pearson, power. Concise and rigorous.

Questions?

Next: Lecture 10 — Classical tests & the LRT framework